GSSSB · STAFF NURSE EXAM

Pharmacology MCQ Bank

400 MCQs · GSSSB pattern · Answer key with explanation



11 Sections | 400 MCQs | Direct + Priority + Scenario | Match & Assertion-Reason | Calculations | Explanations

Hinglish explanation + English exam terms



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MCQ Bank – Sections

1. **General Pharmacology**
   30 MCQs · Definitions, sources, pharmacokinetics (ADME), half-life, bioavailability, pharmacodynamics, receptors, adverse drug reactions, drug laws & schedules, pharmacovigilance
2. **Drug Administration & Dose Calculations**
   30 MCQs · Rights of medication, routes & sites, injection angles, eye/ ear drops, storage, prescription abbreviations, IV drip rate, paediatric dose rules, dilution & unit conversions
3. **Autonomic Nervous System Drugs**
   30 MCQs · Cholinergic & anticholinesterases, organophosphate poisoning, anticholinergics (atropine), adrenergic agonists (adrenaline, dopamine, dobutamine, noradrenaline), α & β blockers
4. **Cardiovascular, Renal & Blood Drugs**
   30 MCQs · Antihypertensives, antianginals, digoxin, antiarrhythmics, diuretics, anticoagulants, antiplatelets, thrombolytics, statins, haematinics, IV fluids, potassium & blood transfusion
5. **Respiratory & Gastrointestinal Drugs**
   30 MCQs · Bronchodilators, inhaled steroids, theophylline, montelukast, antitussives, antihistamines, antacids, PPIs, H. pylori, antiemetics, laxatives, antidiarrhoeals, liver drugs
6. **CNS Drugs, Analgesics & Anaesthetics**
   30 MCQs · Opioids & naloxone, NSAIDs, paracetamol, antiepileptics, antiparkinson drugs, sedative-hypnotics, general & local anaesthetics, neuromuscular blockers
7. **Antimicrobial Drugs**
   30 MCQs · Penicillins, cephalosporins, aminoglycosides, macrolides, tetracyclines, fluoroquinolones, cotrimoxazole, metronidazole, vancomycin, anti-TB, leprosy, antifungal, antiviral, ART, antimalarial, anthelmintic
8. **Endocrine Drugs & Hormones**
   30 MCQs · Insulin types, mixing, sites, storage, hypoglycaemia, oral antidiabetics, thyroid & antithyroid drugs, corticosteroids, contraceptives, oxytocics, tocolytics, pituitary hormones, osteoporosis & gout drugs
9. **Anticancer Drugs, Antidotes & Special Populations**
   30 MCQs · Chemotherapy toxicities & safe handling, immunosuppressants, vaccines & sera, vitamins, antidote master table, teratogens, drugs in pregnancy, children, elderly, renal & hepatic disease, interactions
10. **Emergency Drugs, High-Alert Drugs & Mixed PYQ**
    30 MCQs · Crash-cart drugs, MgSO4, oxytocin, IV fluids, high-alert & LASA drugs, therapeutic drug levels, drug interactions, urine colour clues, drug suffixes, hold-the-drug situations
11. **GSSSB Pattern Mock Test – 100 MCQs (Full Pharmacology Syllabus)**
    100 MCQs · 100 questions, 100 marks, 0.25 negative marking – complete Pharmacology for nurses

Level 1 – Direct: facts, values, definitions

Level 2 – Priority: "first / best / immediate" action

Level 3 – Advanced: scenario, NOT/EXCEPT, match, A-R, calculation



MCQ kaise solve karein?

Pehle khud answer socho, phir options padho. Har section ke baad answer key aur explanation hai, galti wale questions ka number apni notebook me likho. Exam me galat answer par **0.25 marks** kat sakte hain, isliye bina soche guess mat karo.

1 General Pharmacology

Level 1 – Direct

1. Pharmacokinetics ka matlab –
   (a) Drug ki cost (b) Drug body ke saath kya karta hai (c) Body drug ke saath kya karti hai (d) Drug ka source
2. Pharmacodynamics study karta hai –
   (a) Excretion (b) Metabolism (c) Absorption (d) Drug ka mechanism & effect
3. Father of pharmacology –
   (a) Fleming (b) Hippocrates (c) Oswald Schmiedeberg (d) Paul Ehrlich
4. IV route ki bioavailability –
   (a) 25 % (b) 50 % (c) 75 % (d) 100 %
5. Drug metabolism ka main organ –
   (a) Kidney (b) Lung (c) Liver (d) Skin
6. Drug excretion ka main organ –
   (a) Sweat gland (b) Liver (c) Lung (d) Kidney
7. First-pass metabolism avoid hota hai –
   (a) Oral syrup se (b) Oral tablet se (c) Oral capsule se (d) Sublingual route se
8. Half-life (t½) –
   (a) Absorption time (b) Plasma concentration 50 % girne ka time (c) Onset time (d) Drug ka total action time
9. Steady state reach hota hai –
   (a) 1 half-life me (b) 2 half-lives me (c) 4–5 half-lives me (d) 10 half-lives me
10. Loading dose ka purpose –
    (a) Excretion badhana (b) Cost kam karna (c) Toxicity kam karna (d) Jaldi therapeutic level achieve karna
11. Narrow therapeutic index wala drug –
    (a) Penicillin (b) Vitamin C (c) Paracetamol (d) Digoxin
12. Enzyme inducer –
    (a) Rifampicin (b) Cimetidine (c) Ketoconazole (d) Erythromycin
13. Enzyme inhibitor –
    (a) Phenobarbitone (b) Rifampicin (c) Phenytoin (d) Cimetidine
14. Anaphylaxis kaunsa hypersensitivity type hai –
    (a) Type III (b) Type I (IgE) (c) Type II (d) Type IV
15. Tachyphylaxis –
    (a) Addiction (b) Allergy (c) Rapidly developing tolerance (d) Idiosyncrasy
16. Idiosyncrasy –
    (a) Genetically determined abnormal response (b) Tolerance (c) Allergy (d) Dose-related toxicity
17. Inert substance jo psychological effect deta hai –
    (a) Orphan drug (b) Placebo (c) Prodrug (d) Antagonist
18. Rare diseases ke liye drugs –
    (a) OTC drugs (b) Essential drugs (c) Orphan drugs (d) Generic drugs

19. Drugs & Cosmetics Act –
(a) 1940 (b) 1948 (c) 1956 (d) 1985

20. Pharmacovigilance Programme of India (PvPI) start hua –
(a) 1995 (b) 2000 (c) 2010 (d) 2020

Level 2 – Priority & Application

21. Patient ko penicillin injection ke 5 min baad wheezing, hypotension aur urticaria hai – nurse ka pehla action –
(a) Antihistamine oral dena (b) Doctor ko report likhna (c) Injection band karke adrenaline IM dena (d) Patient ko walk karwana

22. Ek drug ka t½ 6 hours hai, 400 mg dose diya – 12 hours baad body me kitna bachega –
(a) 50 mg (b) 100 mg (c) 200 mg (d) 300 mg

23. Liver cirrhosis ke patient me drug dose –
(a) Same rakhni chahiye (b) Kam karni chahiye (highly metabolised drugs) (c) Double karni chahiye (d) IM hi deni chahiye

24. Rifampicin le rahi mahila ko contraception ke liye advice –
(a) Rifampicin band kare (b) Barrier/ non-hormonal method use kare (c) OCP double dose (d) Koi zarurat nahi

25. Prodrug ka example –
(a) Penicillin G (b) Atropine (c) Paracetamol (d) Levodopa

26. Adverse drug reaction ka report karna nurse ka –
(a) Responsibility hai – PvPI ADR form bharna (b) Sirf pharmacist ka kaam (c) Sirf doctor ka kaam (d) Kaam nahi

27. Elderly patient me drug toxicity zyada kyun –
(a) ↓ renal & hepatic clearance (b) ↑ renal flow (c) ↓ body fat (d) ↑ albumin

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Enzyme inducer NAHI hai –
(a) Rifampicin (b) Ketoconazole (c) Carbamazepine (d) Phenytoin

29. Assertion (A): Sublingual nitroglycerin turant kaam karta hai. Reason (R): Sublingual route first-pass metabolism bypass karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Agonist Q. Antagonist R. Partial agonist S. Inverse agonist – 1. Naloxone 2. Morphine 3. Buprenorphine 4. Opposite effect produce karta hai
(a) P-1, Q-2, R-3, S-4 (b) P-2, Q-1, R-3, S-4 (c) P-3, Q-1, R-2, S-4 (d) P-2, Q-3, R-1, S-4

Answer Key & Explanation

1-c Pharmacokinetics = ADME (body → drug).
2-d Pharmacodynamics = drug → body (mechanism, effects).
3-c Oswald Schmiedeberg.
4-d IV me pura drug circulation me – 100 %.
5-c Liver (cytochrome P450).
6-d Kidney.
7-d Sublingual – venous drainage seedha systemic, liver bypass.
8-b t½ = concentration 50 % girne ka time.

9-c 4–5 half-lives.
10-d Loading dose jaldi target level deta hai.
11-d Digoxin (lithium, warfarin, phenytoin, theophylline bhi).
12-a Rifampicin – potent inducer.
13-d Cimetidine – inhibitor.
14-b Type I – IgE mediated.
15-c Tachyphylaxis – jaldi tolerance (ephedrine).
16-a Idiosyncrasy – genetic.
17-b Placebo.
18-c Orphan drugs.
19-a 1940.
20-c 2010 (IPC Ghaziabad).
21-c Anaphylaxis – stop drug, adrenaline 0.5 mg IM.
22-b 2 half-lives → 400 → 200 → 100 mg.
23-b Hepatic clearance ↓ – dose kam.
24-b Rifampicin enzyme induction se OCP fail – barrier method.
25-d Levodopa → dopamine (prodrug).
26-a ADR reporting – nurse bhi report karti hai.
27-a Elderly me renal/ hepatic clearance kam.
28-b Ketoconazole enzyme inhibitor hai.
29-a Dono sahi, R reason hai.
30-b Morphine agonist, naloxone antagonist, buprenorphine partial agonist.

2 Drug Administration & Dose Calculations

? Level 1 – Direct

1. Medication dene se pehle patient identification ke liye minimum identifiers –
   (a) 1 (b) 2 (c) 3 (d) 4
2. Adults me IM injection ka safest site –
   (a) Deltoid (b) Ventrogluteal (c) Dorsogluteal (d) Forearm
3. Infants me IM injection ka site of choice –
   (a) Vastus lateralis (b) Deltoid (c) Dorsogluteal (d) Abdomen
4. Deltoid me maximum volume –
   (a) 1 ml (b) 3 ml (c) 4 ml (d) 5 ml
5. Intradermal injection ka angle –
   (a) 10–15° (b) 45° (c) 60° (d) 90°
6. IM injection ka angle –
   (a) 10° (b) 15° (c) 30° (d) 90°
7. Mantoux test ka route –
   (a) Subcutaneous (b) IV (c) IM (d) Intradermal
8. Z-track technique ka use –
   (a) Iron dextran IM (b) Heparin (c) BCG (d) Insulin
9. Heparin SC dete waqt –
   (a) Deltoid me dena (b) Aspirate karna (c) Aspirate & massage nahi karna (d) Site massage karna
10. "STAT" ka matlab –
    (a) Twice daily (b) As needed (c) At bedtime (d) Immediately
11. "PRN" ka matlab –
    (a) Before meals (b) Every morning (c) As needed (d) After meals
12. "HS" ka matlab –
    (a) Twice daily (b) Every hour (c) At bedtime (d) Thrice daily
13. Eye drops kahan daalte hain –
    (a) Lower conjunctival sac (b) Inner canthus par (c) Cornea par seedha (d) Upper lid
14. Adult me ear drops ke liye pinna –
    (a) Up & back (b) Up & forward (c) Down & forward (d) Down & back
15. 3 saal se chhote bachche me pinna –
    (a) Up & forward (b) Up & back (c) Down & back (d) Seedha
16. 1 teaspoon =
    (a) 2 ml (b) 5 ml (c) 10 ml (d) 15 ml
17. 1 tablespoon =
    (a) 5 ml (b) 10 ml (c) 15 ml (d) 30 ml
18. Micro-drip (paediatric) set ka drop factor –
    (a) 10 drops/ml (b) 15 drops/ml (c) 20 drops/ml (d) 60 drops/ml

19. Nitroglycerin tablets store karte hain –
(a) Fridge freezer me (b) Dark glass bottle me (c) Plastic pouch me (d) Open container me

20. Medication error hone par nurse ka pehla kadam –
(a) Next dose skip karna (b) Chupa lena (c) Patient ka assessment & doctor ko inform (d) Incident report baad me

? Level 2 – Priority & Application

21. 1000 ml NS 8 hours me dena hai, drop factor 15 drops/ml – drops/min –
(a) 21 (b) 31 (c) 42 (d) 62

22. 500 ml DNS 4 hours me, drop factor 20 – drops/min –
(a) 21 (b) 31 (c) 42 (d) 63

23. Order: 250 mg; available 500 mg/ 2 ml – kitna ml dena hai –
(a) 0.5 ml (b) 1 ml (c) 2 ml (d) 4 ml

24. Order: amoxicillin 40 mg/kg/day in 2 divided doses, child 15 kg – per dose –
(a) 150 mg (b) 300 mg (c) 400 mg (d) 600 mg

25. Young's rule se 6 saal ke bachche ki dose (adult dose 300 mg) –
(a) 50 mg (b) 100 mg (c) 150 mg (d) 200 mg

26. Microdrip set se 30 ml/hr chalana hai – drops/min –
(a) 10 (b) 15 (c) 30 (d) 60

27. Patient ne kaha "ye goli roj wali jaisi nahi lag rahi" – nurse kya kare –
(a) Goli tod kar de (b) Dena rok kar order & drug recheck kare (c) Ignore kare (d) Patient ko samjha kar de de

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Heparin 25,000 units in 500 ml; order 1000 units/hr – ml/hr –
(a) 10 (b) 20 (c) 25 (d) 40

29. Assertion (A): Insulin aur heparin site par massage nahi karna chahiye. Reason (R): Massage se absorption change hota hai aur heparin me haematoma ban sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Medication administration ke rights me NAHI aata –
(a) Right documentation (b) Right route (c) Right price (d) Right time

Answer Key & Explanation

1-b 2 identifiers (naam + UHID/ DOB) – room number nahi.
2-b Ventrogluteal – nerves/ vessels se door.
3-a Vastus lateralis.
4-a Deltoid – 1 ml (max ~2 ml).
5-a ID – 10–15°.
6-d IM – 90°.
7-d Mantoux 0.1 ml ID.
8-a Z-track – iron dextran (skin staining rokta hai).
9-c Heparin – na aspirate, na massage (haematoma).
10-d STAT – turant.
11-c PRN – zarurat pe.
12-c HS – hora somni, bedtime.
13-a Lower conjunctival sac; phir inner canthus dabana.
14-a Adult – up & back.
15-c < 3 yrs – down & back.
16-b 5 ml.
17-c 15 ml.
18-d Micro-drip 60.
19-b Light & moisture se kharab – dark glass.
20-c Pehle patient safety – assess, inform, phir incident

report.

21-b 1000 × 15 / 480 = 31.25 ≈ 31.

22-c 500 × 20 / 240 = 41.7 ≈ 42.

23-b 250/500 × 2 = 1 ml.

24-b 40 × 15 = 600 mg/day ÷ 2 = 300 mg.

25-b 6/(6+12) × 300 = 100 mg.

26-c Microdrip me ml/hr = drops/min.

27-b Patient ki shanka par ruk kar verify karo.

28-b 25,000/500 = 50 units/ml → 1000/50 = 20 ml/hr.

29-a Dono sahi, R reason hai.

30-c "Right price" koi right nahi.

3 Autonomic Nervous System Drugs

? Level 1 – Direct

1. Parasympathetic system ka neurotransmitter –
   (a) Acetylcholine (b) Dopamine (c) Serotonin (d) Noradrenaline
2. Myasthenia gravis ka drug –
   (a) Atropine (b) Pyridostigmine (c) Propranolol (d) Succinylcholine
3. Myasthenia gravis diagnosis test –
   (a) Schick (b) Edrophonium (Tensilon) test (c) Dick test (d) Mantoux
4. Glaucoma me miotic drug –
   (a) Pilocarpine (b) Atropine (c) Tropicamide (d) Adrenaline
5. Organophosphate poisoning ka antidote –
   (a) Atropine + pralidoxime (b) Naloxone (c) Vitamin K (d) Flumazenil
6. Cholinergic crisis ke features –
   (a) Tachycardia & dry skin (b) SLUDGE (salivation, lacrimation, urination…) (c) Dry mouth (d) Mydriasis
7. Atropine ka effect –
   (a) Miosis (b) Pupil dilation (mydriasis) (c) ↑ secretions (d) Bradycardia
8. Atropine contraindicated –
   (a) OP poisoning (b) Narrow-angle glaucoma (c) Pre-anaesthesia (d) Bradycardia
9. Anticholinergic poisoning ka antidote –
   (a) Protamine (b) Physostigmine (c) Atropine (d) Naloxone
10. Anaphylaxis me adrenaline ki dose (adult) –
    (a) 0.05 mg oral (b) 0.5 mg IM (1:1000) (c) 1 mg IV (1:1000) (d) 5 mg SC
11. Cardiac arrest me adrenaline –
    (a) 0.1 mg SC (b) 0.5 mg IM (c) 1 mg IV (1:10,000) every 3–5 min (d) 10 mg IV
12. Low dose dopamine ka effect –
    (a) Vasoconstriction (b) Renal vasodilation (c) Bronchospasm (d) Bradycardia
13. Septic shock me first-line vasopressor –
    (a) Dopamine (b) Atropine (c) Noradrenaline (d) Isoprenaline
14. Cardiogenic shock me inotrope –
    (a) Atropine (b) Dobutamine (c) Neostigmine (d) Propranolol
15. Noradrenaline extravasation ka antidote –
    (a) Hyaluronidase (b) Calcium (c) Phentolamine (d) Atropine
16. Non-selective β-blocker –
    (a) Bisoprolol (b) Propranolol (c) Atenolol (d) Metoprolol
17. β-blocker contraindicated –
    (a) Bronchial asthma (b) Migraine (c) Angina (d) Hypertension
18. BPH me use hone wala α1-blocker –
    (a) Tamsulosin (b) Propranolol (c) Atropine (d) Salbutamol

19. Prazosin ka important side effect –
(a) Bradycardia (b) First-dose hypotension (c) Cough (d) Hyperkalaemia

20. Symptomatic bradycardia ka drug –
(a) Propranolol (b) Digoxin (c) Verapamil (d) Atropine

? Level 2 – Priority & Application

21. OP poisoning me atropine kab tak dete hain –
(a) Pupils pin-point hone tak (b) Jab tak patient so jaye (c) Sirf ek dose (d) Atropinisation tak (dry chest/ secretions band, HR > 80)

22. Diabetic patient propranolol le raha hai – nurse kya sikhaye –
(a) Kuch nahi (b) Hypoglycaemia ke signs (tremor, palpitation) mask ho sakte hain (c) Sugar badhega (d) Insulin band karo

23. β-blocker dene se pehle check karna –
(a) Urine colour (b) Weight (c) Pulse & BP (d) Temperature

24. Myasthenic patient me weakness ↑ aur excessive secretions – likely –
(a) Cholinergic crisis (overdose) (b) Myasthenic crisis (c) Anaphylaxis (d) Hypoglycaemia

25. Post-op urinary retention (non-obstructive) me –
(a) Neostigmine / bethanechol (b) Morphine (c) Propranolol (d) Atropine

26. Pre-anaesthetic atropine dene ka reason –
(a) Sedation (b) Pain relief (c) Secretions kam karna & vagal bradycardia rokna (d) Muscle relaxation

27. Abrupt β-blocker stop karne se –
(a) Rebound tachycardia/ angina (b) Asthma (c) Hypoglycaemia (d) Bradycardia

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Atropine toxicity ka feature NAHI hai –
(a) Dry mouth (b) Hyperthermia (c) Delirium (d) Excessive salivation

29. Assertion (A): Pralidoxime OP poisoning me jaldi dena chahiye. Reason (R): "Aging" ke baad enzyme reactivate nahi hota.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Adrenaline 1:1000 ka matlab –
(a) 0.01 mg/ml (b) 0.1 mg/ml (c) 1 mg/ml (d) 10 mg/ml

Answer Key & Explanation

1-a ACh (sympathetic postganglionic – noradrenaline).
2-b Pyridostigmine – anticholinesterase.
3-b Edrophonium – short acting.
4-a Pilocarpine – miosis, ↑ aqueous outflow.
5-a Atropine (muscarinic) + pralidoxime (enzyme reactivator).
6-b SLUDGE + miosis + bradycardia + bronchorrhoea.
7-b Mydriasis, tachycardia, dry secretions.
8-b Angle closure badh sakta hai.
9-b Physostigmine.
10-b 0.5 mg IM anterolateral thigh.
11-c 1 mg IV 1:10,000.
12-b Low dose – D1 renal vasodilation.
13-c Noradrenaline.
14-b Dobutamine – β1.
15-c Phentolamine infiltration.
16-b Propranolol.
17-a β2 block – bronchospasm.
18-a Tamsulosin – uroselective.
19-b Bedtime par pehli dose.

20-d Atropine 1 mg IV (latest ACLS).
21-d Atropinisation end-point.
22-b β-blocker adrenergic signs mask karta hai (sweating nahi).
23-c Apical pulse < 60 / low BP – hold.
24-a Excess secretions = cholinergic crisis.
25-a Cholinergic drugs bladder contract karte hain.
26-c Antisialagogue + vagolytic.
27-a Rebound – taper karna.
28-d Atropine se dry mouth hota hai, salivation nahi.
29-a Dono sahi, R reason hai.
30-c 1 g/1000 ml = 1 mg/ml.

4 Cardiovascular, Renal & Blood Drugs

Level 1 – Direct

1. ACE inhibitor ka common side effect –
   (a) Dry cough (b) Bradycardia (c) Ankle oedema (d) Constipation
2. Amlodipine ka common side effect –
   (a) Dry cough (b) Hyperkalaemia (c) Gynaecomastia (d) Ankle oedema
3. Pregnancy me contraindicated antihypertensive –
   (a) Nifedipine (b) Methyldopa (c) Labetalol (d) Enalapril
4. Sodium nitroprusside ki toxicity –
   (a) Cyanide toxicity (b) Nephrotoxicity (c) Ototoxicity (d) Hypoglycaemia
5. Clonidine abruptly band karne se –
   (a) Hypoglycaemia (b) Hypotension (c) Rebound hypertension (d) Diarrhoea
6. Sublingual nitroglycerin – angina me max tablets –
   (a) 1 tablet only (b) 3 tablets, 5 min apart (c) 5 tablets, 1 min apart (d) 10 tablets
7. Nitrates ka common side effect –
   (a) Bradycardia (b) Headache (c) Constipation (d) Cough
8. Digoxin dene se pehle check –
   (a) Blood sugar (b) Temperature (c) Weight (d) Apical pulse 1 full minute
9. Digoxin toxicity ka early sign –
   (a) Diarrhoea only (b) Fever (c) Anorexia, nausea, vomiting (d) Hypertension
10. Digoxin toxicity ka antidote –
    (a) Digoxin-specific Fab (b) Protamine (c) Vitamin K (d) Naloxone
11. Furosemide kahan act karti hai –
    (a) DCT (b) PCT (c) Collecting duct (d) Loop of Henle
12. Loop diuretic ka important side effect –
    (a) Hyperkalaemia (b) Hypokalaemia (c) Hypercalcaemia (d) Hypoglycaemia
13. Potassium-sparing diuretic –
    (a) Hydrochlorothiazide (b) Spironolactone (c) Mannitol (d) Furosemide
14. Heparin ka antidote –
    (a) Naloxone (b) Vitamin K (c) Protamine sulphate (d) Desferrioxamine
15. Warfarin monitoring test –
    (a) aPTT (b) Platelet count (c) PT/ INR (d) Bleeding time
16. Heparin monitoring test –
    (a) ESR (b) Hb (c) INR (d) aPTT
17. Warfarin ka antidote –
    (a) Vitamin K (b) Calcium (c) Atropine (d) Protamine
18. Statins kab dena better (simvastatin) –
    (a) Raat ko (b) Koi farak nahi (c) Dopahar (d) Subah khali pet

19. Statin ka serious side effect –
(a) Orange urine (b) Cough (c) Myopathy/ rhabdomyolysis (d) Ankle oedema

20. Oral iron ka side effect –
(a) Hair loss (b) Black stools & constipation (c) Diarrhoea always (d) Red urine

? Level 2 – Priority & Application

21. Digoxin dene se pehle apical pulse 52/min hai – nurse –
(a) Dose de de (b) Double dose de (c) Dose hold kare & doctor ko inform kare (d) Atropine de

22. Furosemide + digoxin le rahe patient me sabse zyada risk –
(a) Hypertension (b) Hypoglycaemia (c) Hypokalaemia se digoxin toxicity (d) Hyperkalaemia

23. KCl IV dene ka sahi tarika –
(a) SC (b) IV bolus (c) Diluted infusion, kabhi IV push nahi (d) IM

24. Blood transfusion ke pehle 15 min me patient ko chills, back pain, hypotension – nurse ka pehla action –
(a) Bag change karna (b) Rate badhana (c) Transfusion turant band karna & NS se line open rakhna (d) Antipyretic dekar continue

25. Oral iron syrup dete waqt advice –
(a) Doodh ke saath (b) Antacid ke saath (c) Straw se lena (teeth staining) (d) Chai ke saath

26. Warfarin le raha patient – diet advice –
(a) Bilkul band (b) Double karna (c) Koi farak nahi (d) Green leafy veg ka intake consistent rakhna

27. Thrombolytic therapy (MI) ka main risk –
(a) Hyperglycaemia (b) Bleeding (intracranial) (c) Hypertension (d) Constipation

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Hyperkalaemia NAHI karta –
(a) Spironolactone (b) Furosemide (c) Amiloride (d) Enalapril

29. Assertion (A): Warfarin start karte waqt heparin kuch din saath me dete hain. Reason (R): Warfarin ka full effect 3–5 din me aata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Amiodarone Q. Hydralazine R. Adenosine S. Streptokinase – 1. Lupus-like syndrome 2. Pulmonary fibrosis/ thyroid 3. PSVT, very short t½ 4. Antigenic, repeat nahi
(a) P-2, Q-1, R-3, S-4 (b) P-4, Q-1, R-3, S-2 (c) P-2, Q-3, R-1, S-4 (d) P-1, Q-2, R-3, S-4

✓ Answer Key & Explanation

1-a Bradykinin ↑ – dry cough, angioedema.
2-d Pedal oedema (vasodilation).
3-d ACEI – fetal renal damage.
4-a Cyanide; light se protect.
5-c Rebound HTN – taper.
6-b 3 tabs 5 min apart; relief na ho to emergency.
7-b Vasodilation – headache.
8-d Apical pulse < 60 → hold.
9-c GI symptoms early; yellow vision bhi.
10-a Digibind.
11-d Thick ascending loop.
12-b Hypokalaemia.
13-b Spironolactone.
14-c Protamine.
15-c INR 2–3.
16-d aPTT 1.5–2.5 × control.
17-a Vitamin K.
18-a Cholesterol synthesis raat ko zyada.

19-c Muscle pain report karo – CPK.
20-b Black stools normal hain.
21-c < 60 – hold.
22-c Low K^+ digoxin toxicity badhata hai.
23-c KCl IV push = cardiac arrest.
24-c Acute haemolytic reaction – stop turant.
25-c Straw se – teeth stain.
26-d Vitamin K consistent – INR stable.
27-b Bleeding.
28-b Furosemide hypokalaemia karti hai.
29-a Dono sahi, R reason hai.
30-a Amiodarone–fibrosis/thyroid, hydralazine–lupus, adenosine–PSVT, SK–antigenic.

5 Respiratory & Gastrointestinal Drugs

? Level 1 – Direct

1. Acute asthma attack ka DOC –
(a) Budesonide (b) Salbutamol (c) Sodium cromoglycate (d) Montelukast

2. Asthma ka sabse effective long-term controller –
(a) Theophylline (b) Salbutamol (c) Inhaled corticosteroid (d) Antihistamine

3. Inhaled steroid ke baad mouth rinse ka reason –
(a) Oral candidiasis rokna (b) Tooth decay (c) Cough (d) Taste

4. COPD me preferred long-acting bronchodilator –
(a) Tiotropium (b) Cromolyn (c) Propranolol (d) Atropine IV

5. Theophylline ka therapeutic range –
(a) 1–2 µg/ml (b) 10–20 µg/ml (c) 30–40 µg/ml (d) 50–100 µg/ml

6. Salbutamol ka side effect –
(a) Miosis (b) Constipation (c) Tremor & tachycardia (d) Bradycardia

7. Sirf prophylaxis (acute me useless) –
(a) Ipratropium (b) Adrenaline (c) Salbutamol (d) Sodium cromoglycate

8. Leukotriene antagonist –
(a) Ranitidine (b) Montelukast (c) Ondansetron (d) Cetirizine

9. Mucolytic + paracetamol antidote –
(a) Dextromethorphan (b) N-acetylcysteine (c) Codeine (d) Guaifenesin

10. Non-sedating antihistamine –
(a) Cetirizine (b) Promethazine (c) Diphenhydramine (d) Chlorpheniramine

11. Diarrhoea karne wala antacid –
(a) Sucralfate (b) Calcium carbonate (c) Aluminium hydroxide (d) Magnesium hydroxide

12. Constipation karne wala antacid –
(a) Magnesium hydroxide (b) Bisacodyl (c) Aluminium hydroxide (d) Lactulose

13. PPI dene ka best time –
(a) Khane ke saath (b) Raat ko khane ke baad (c) Nashte se 30–60 min pehle (d) Kabhi bhi

14. Sabse potent acid suppressant –
(a) Sucralfate (b) Antacid (c) Omeprazole (d) Ranitidine

15. NSAID-induced ulcer prevention & pregnancy me contraindicated –
(a) Omeprazole (b) Famotidine (c) Sucralfate (d) Misoprostol

16. Chemotherapy-induced vomiting ka DOC –
(a) Ondansetron (b) Promethazine (c) Domperidone (d) Loperamide

17. Metoclopramide ka side effect –
(a) Acute dystonia (EPS) (b) Hypertension (c) Hearing loss (d) Constipation

18. Hepatic encephalopathy me laxative –
(a) Lactulose (b) Senna (c) Liquid paraffin (d) Castor oil

19. Safest & most physiological laxative –
(a) Castor oil (b) Bisacodyl (c) Bulk-forming (isabgol) (d) Magnesium sulphate

20. Morning sickness ke liye –
(a) Misoprostol (b) Metoclopramide IV (c) Ondansetron high dose (d) Doxylamine + pyridoxine

? Level 2 – Priority & Application

21. Patient ko salbutamol aur budesonide inhaler dono hai – sequence –
(a) Pehle budesonide (b) Koi farak nahi (c) Pehle salbutamol, 5 min baad budesonide (d) Dono ek saath

22. Aminophylline IV dete waqt –
(a) Slowly (≥ 20 min) (b) SC (c) IM (d) IV bolus fast

23. Ciprofloxacin + antacid le raha patient – advice –
(a) Saath me lo (b) Antacid band (c) 2 hours ka gap rakho (d) Doodh ke saath lo

24. Isabgol lene wale patient ko –
(a) Sukha lena (b) Bhar-poor paani ke saath lena (c) Doodh me ubaalna (d) Raat ko bina paani

25. Abdominal pain ke saath suspected appendicitis – laxative –
(a) Contraindicated hai (b) Castor oil (c) Bisacodyl de do (d) Enema de do

26. Liquid paraffin elderly bedtime me avoid kyun –
(a) Diarrhoea (b) Hypertension (c) Hypoglycaemia (d) Aspiration – lipoid pneumonia

27. 2 saal ke bachche ko dysentery (bloody stool) – loperamide –
(a) Sirf raat ko (b) Double dose (c) Nahi dena chahiye (d) Turant dena chahiye

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Theophylline level badhane wala NAHI hai –
(a) Cimetidine (b) Erythromycin (c) Ciprofloxacin (d) Smoking

29. Assertion (A): Lactulose hepatic encephalopathy me dete hain. Reason (R): Lactulose colon ko acidic karke ammonia absorption kam karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. H. pylori triple therapy me NAHI hota –
(a) PPI (b) Amoxicillin (c) Ondansetron (d) Clarithromycin

✓ Answer Key & Explanation

1-b SABA – reliever.
2-c ICS – controller.
3-a Oral thrush; hoarseness bhi.
4-a LAMA – tiotropium.
5-b 10–20 µg/ml.
6-c β2 effects – tremor, tachycardia, ↓ K^+.
7-d Mast cell stabiliser.
8-b Montelukast.
9-b NAC.
10-a 2nd gen.
11-d Mg – diarrhoea.
12-c Al – constipation.
13-c Empty stomach, before breakfast.
14-c PPI.
15-d Misoprostol – abortifacient.
16-a 5-HT3 antagonist.
17-a D2 block – EPS.
18-a Lactulose.
19-c Bulk-forming.
20-d Doxylamine-pyridoxine.
21-c Bronchodilator pehle – steroid ki deep penetration.
22-a Fast IV – arrhythmia, seizures.
23-c Chelation – gap.

24-b Paani kam – obstruction.
25-a Perforation ka risk.
26-d Aspiration.
27-c Toxic megacolon risk.
28-d Smoking theophylline level kam karta hai (enzyme induction).
29-a Dono sahi, R reason hai.
30-c Ondansetron antiemetic hai.

6 CNS Drugs, Analgesics & Anaesthetics

Level 1 – Direct

1. Morphine ka sabse dangerous side effect –
   (a) Respiratory depression (b) Nausea (c) Constipation (d) Miosis
2. Morphine ka woh side effect jisme tolerance nahi hota –
   (a) Constipation (b) Respiratory depression (c) Euphoria (d) Analgesia
3. Opioid overdose ka antidote –
   (a) Naloxone (b) NAC (c) Atropine (d) Flumazenil
4. Pin-point pupils –
   (a) Amphetamine (b) Cocaine (c) Atropine (d) Morphine overdose
5. Paracetamol ka adult max daily dose –
   (a) 1 g (b) 4 g (c) 8 g (d) 10 g
6. Paracetamol poisoning ka antidote –
   (a) N-acetylcysteine (b) Vitamin K (c) Naloxone (d) Atropine
7. Bachchon me aspirin avoid – kyunki –
   (a) Kernicterus (b) Teeth staining (c) Reye's syndrome (d) Gray baby
8. NSAIDs ka common side effect –
   (a) Gastric ulcer (b) Hair loss (c) Hypoglycaemia (d) Hypokalaemia
9. Phenytoin ka characteristic side effect –
   (a) Weight loss (b) Gum hyperplasia (c) Hypertension (d) Ototoxicity
10. Phenytoin IV kis fluid me –
    (a) Ringer lactate (b) 5 % dextrose (c) DNS (d) Normal saline
11. Pregnancy me neural tube defect karne wala antiepileptic –
    (a) Phenobarbitone (b) Lamotrigine (c) Valproate (d) Levetiracetam
12. Absence seizures ka DOC –
    (a) Ethosuximide (b) Phenytoin (c) Phenobarbitone (d) Carbamazepine
13. Status epilepticus me first drug –
    (a) Paracetamol (b) Lorazepam IV (c) Phenytoin oral (d) Valproate oral
14. Trigeminal neuralgia ka DOC –
    (a) Aspirin (b) Carbamazepine (c) Morphine (d) Diazepam
15. Levodopa ke saath carbidopa ka purpose –
    (a) Toxicity badhana (b) Peripheral breakdown rokna (c) Constipation (d) Sedation
16. Benzodiazepine overdose antidote –
    (a) Flumazenil (b) NAC (c) Physostigmine (d) Naloxone
17. Malignant hyperthermia ka drug –
    (a) Succinylcholine (b) Paracetamol (c) Dantrolene (d) Halothane
18. Depolarising muscle relaxant –
    (a) Atracurium (b) Vecuronium (c) Succinylcholine (d) Rocuronium

19. Dissociative anaesthetic –
(a) Ketamine (b) Halothane (c) Thiopentone (d) Propofol

20. Local anaesthetic + adrenaline contraindicated –
(a) Abdomen (b) Back (c) Fingers, toes, penis (end arteries) (d) Thigh

? Level 2 – Priority & Application

21. Morphine dene se pehle respiratory rate 10/min – nurse –
(a) Dose de de (b) Half dose (c) Dose hold & doctor inform (d) Naloxone se pehle morphine

22. Naloxone dene ke baad patient ko monitor karna zaruri kyun –
(a) Hypertension (b) Hypoglycaemia (c) Naloxone short acting – re-narcotisation ho sakta hai (d) Allergy

23. Spinal anaesthesia ke baad headache – nurse advice –
(a) Flat lie, fluids badhao (b) Turant chalna (c) Head end up 90° (d) Fluids restrict

24. Long-term morphine patient ke liye routine order –
(a) Diuretic (b) Antacid (c) Laxative/ stool softener (d) Antidiarrhoeal

25. Phenytoin le rahe patient ko advice –
(a) Oral hygiene & gum care (b) Daant brush band (c) Drug khud band kar sakte (d) Dairy band

26. Levodopa ke saath avoid –
(a) Pyridoxine (vitamin B6) supplements (b) Vitamin C (c) Water (d) Calcium

27. Diazepam IV ke baad monitor –
(a) Urine colour (b) Weight (c) Respiration & BP (d) Blood sugar

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Lignocaine (without adrenaline) max dose ~4.5 mg/kg; 60 kg patient ke liye max –
(a) 180 mg (b) 270 mg (c) 450 mg (d) 600 mg

29. Assertion (A): Succinylcholine burns/ crush injury me avoid karte hain. Reason (R): Ye severe hyperkalaemia kar sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Carbamazepine Q. Valproate R. Phenytoin S. Phenobarbitone – 1. SJS (HLA-B*1502), hyponatraemia 2. Hepatotoxicity, NTD 3. Gum hyperplasia, nystagmus 4. Neonatal seizures DOC
(a) P-3, Q-2, R-1, S-4 (b) P-1, Q-3, R-2, S-4 (c) P-2, Q-1, R-3, S-4 (d) P-1, Q-2, R-3, S-4

✓ Answer Key & Explanation

1-a Respiratory depression.
2-a Constipation & miosis me tolerance nahi.
3-a Naloxone.
4-d Opioid – miosis.
5-b 4 g/day.
6-a NAC – glutathione replenish.
7-c Reye's – viral infection ke saath.
8-a PG inhibition – gastric ulcer.
9-b Gum hyperplasia, hirsutism.
10-d Dextrose me precipitate hota hai.
11-c Valproate – NTD.
12-a Ethosuximide.
13-b BZD first-line.
14-b Carbamazepine.
15-b Dopa decarboxylase inhibitor.
16-a Flumazenil.
17-c Dantrolene.
18-c Sch – depolarising.
19-a Ketamine.
20-c End-arteries – gangrene risk.
21-c RR < 12 – hold.
22-c Naloxone ka effect morphine se jaldi khatam.

23-a PDPH – flat, fluids.
24-c Constipation me tolerance nahi.
25-a Gum hyperplasia.
26-a B6 levodopa ka peripheral breakdown badhata hai (carbidopa ke bina).
27-c Respiratory depression, hypotension.
28-b 4.5 × 60 = 270 mg.
29-a Dono sahi, R reason hai.
30-d Sahi match.

7 Antimicrobial Drugs

Level 1 – Direct

1. Penicillin ki khoj –
   (a) Louis Pasteur (b) Alexander Fleming (c) Edward Jenner (d) Robert Koch
2. Penicillin ka sabse dangerous reaction –
   (a) Nausea (b) Rash (c) Anaphylaxis (d) Diarrhoea
3. Rheumatic fever prophylaxis –
   (a) Gentamicin monthly (b) Benzathine penicillin har 21 din (c) Ceftriaxone weekly (d) Amoxicillin daily
4. β-lactamase inhibitor –
   (a) Trimethoprim (b) Cilastatin (c) Clavulanic acid (d) Probenecid
5. Surgical prophylaxis ka common antibiotic –
   (a) Vancomycin (b) Colistin (c) Meropenem (d) Cefazolin
6. Aminoglycosides ki main toxicity –
   (a) Hepatotoxicity (b) Teeth staining (c) Cardiotoxicity (d) Nephrotoxicity & ototoxicity
7. 8 saal se chhote bachchon me avoid –
   (a) Azithromycin (b) Tetracycline (c) Cefixime (d) Amoxicillin
8. Gray baby syndrome –
   (a) Chloramphenicol (b) Tetracycline (c) Gentamicin (d) Penicillin
9. Tendon rupture –
   (a) Amoxicillin (b) Metronidazole (c) Ciprofloxacin (d) Erythromycin
10. Metronidazole + alcohol –
    (a) Disulfiram-like reaction (b) Hypertension (c) Hypoglycaemia (d) Kuch nahi
11. Amoebiasis ka DOC –
    (a) Chloroquine (b) Metronidazole (c) Albendazole (d) Penicillin
12. Red man syndrome –
    (a) Gentamicin (b) Rifampicin (c) Ceftriaxone (d) Vancomycin
13. MRSA ka DOC –
    (a) Ampicillin (b) Penicillin G (c) Cefixime (d) Vancomycin
14. INH ke saath pyridoxine dene ka reason –
    (a) Hepatitis rokna (b) Orange urine (c) Gout (d) Peripheral neuropathy rokna
15. Orange-red urine –
    (a) Ethambutol (b) Rifampicin (c) Pyrazinamide (d) INH
16. Optic neuritis (colour vision) –
    (a) Ethambutol (b) INH (c) Rifampicin (d) Streptomycin
17. Hyperuricaemia/ gout –
    (a) INH (b) Rifampicin (c) Pyrazinamide (d) Ethambutol
18. Oral thrush ke liye (swish & swallow) –
    (a) Griseofulvin (b) Acyclovir (c) Nystatin (d) Amphotericin IV

19. Herpes simplex/ zoster ka drug –
(a) Acyclovir (b) Oseltamivir (c) Zidovudine (d) Fluconazole

20. Scabies ka DOC –
(a) Nystatin (b) Acyclovir (c) Albendazole (d) Permethrin 5 %

? Level 2 – Priority & Application

21. Antibiotic ka first dose dene se pehle –
(a) Diet chart (b) X-ray (c) Weight only (d) Allergy history & culture sample

22. Tetracycline le rahe patient ko advice –
(a) Iron ke saath (b) Sote waqt (c) Paani ke saath, doodh/ antacid ke saath nahi, letne se pehle nahi (d) Doodh ke saath

23. Gentamicin le rahe patient me report karna –
(a) Gum hyperplasia (b) Black stool (c) Tinnitus/ ghatta sunai dena (d) Orange urine

24. Vancomycin infusion –
(a) IM (b) IV push (c) Kam se kam 60 min me slowly (d) 5 min me

25. Cotrimoxazole le rahe patient ko –
(a) Doodh band (b) Bhar-poor paani piyo (crystalluria) (c) Fluid restrict (d) Dhoop me raho

26. Amphotericin B infusion me chills & fever – nurse –
(a) Premedication (paracetamol/ antihistamine) as ordered, rate slow (b) Infusion fast (c) Ice bath (d) Ignore

27. Primaquine dene se pehle test –
(a) ECG (b) G6PD (c) LFT (d) HbA1c

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Bactericidal NAHI hai –
(a) Penicillin (b) Tetracycline (c) Ciprofloxacin (d) Gentamicin

29. Assertion (A): Rifampicin le rahi mahila me OCP fail ho sakti hai. Reason (R): Rifampicin enzyme inducer hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Zidovudine Q. Tenofovir R. Nevirapine S. Efavirenz – 1. Anaemia 2. Nephrotoxicity 3. Rash/ hepatotoxicity 4. Vivid dreams (CNS)
(a) P-2, Q-1, R-3, S-4 (b) P-1, Q-2, R-3, S-4 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-3, R-2, S-4

✓ Answer Key & Explanation

1-b Fleming 1928.
2-c Anaphylaxis.
3-b 1.2 MU har 21 din.
4-c Clavulanic acid.
5-d Cefazolin incision se 60 min pehle.
6-d Nephro- & ototoxicity.
7-b Teeth staining.
8-a Chloramphenicol.
9-c Fluoroquinolones.
10-a Disulfiram-like.
11-b Metronidazole.
12-d Fast infusion – histamine release.
13-d Vancomycin.
14-d Pyridoxine.
15-b Rifampicin.
16-a Ethambutol.
17-c Pyrazinamide.
18-c Nystatin.
19-a Acyclovir.
20-d Permethrin.

21-d Allergy & C/S.
22-c Chelation & oesophageal ulcer.
23-c Ototoxicity.
24-c Slow infusion.
25-b Crystalluria rokna.
26-a Infusion reaction – premedicate, slow.
27-b G6PD deficiency – haemolysis.
28-b Tetracycline bacteriostatic.
29-a Dono sahi, R reason hai.
30-b Sahi match.

8 Endocrine Drugs & Hormones

Level 1 – Direct

1. Insulin ki khoj –
   (a) Banting & Best (b) Jenner (c) Fleming (d) Pasteur
2. U-100 insulin me –
   (a) 10 units/ml (b) 40 units/ml (c) 100 units/ml (d) 1000 units/ml
3. IV di ja sakne wali insulin –
   (a) NPH (b) Regular (short-acting) (c) Premixed 30/70 (d) Glargine
4. Peakless (basal) insulin –
   (a) Glargine (b) NPH (c) Lispro (d) Regular
5. Cloudy insulin –
   (a) Glargine (b) NPH (c) Regular (d) Lispro
6. Insulin mixing me pehle kaunsi khinchte hain –
   (a) Clear (regular) (b) Cloudy (NPH) (c) Koi bhi (d) Glargine
7. Insulin ka fastest absorption site –
   (a) Buttock (b) Thigh (c) Abdomen (d) Arm
8. Injection site rotate karne ka reason –
   (a) Pain badhana (b) Infection (c) Lipodystrophy rokna (d) Cost
9. Unopened insulin storage –
   (a) Dhoop me (b) Freezer (c) 2–8 °C (fridge, freezer nahi) (d) 40 °C
10. Insulin ka sabse common side effect –
    (a) Hypoglycaemia (b) Hair loss (c) Hyperkalaemia (d) Weight loss
11. Hypoglycaemia ki cut-off –
    (a) < 70 mg/dl (b) < 100 mg/dl (c) < 120 mg/dl (d) < 150 mg/dl
12. Type 2 DM ka first-line drug –
    (a) Insulin (b) Glibenclamide (c) Metformin (d) Acarbose
13. Metformin ka serious side effect –
    (a) Hyperkalaemia (b) Hypoglycaemia (alone) (c) Weight gain (d) Lactic acidosis
14. Weight gain & hypoglycaemia karne wala OHA –
    (a) Metformin (b) Acarbose (c) Sitagliptin (d) Glibenclamide
15. Heart failure me avoid –
    (a) Metformin (b) Pioglitazone (c) Insulin (d) Empagliflozin
16. Levothyroxine kab lena –
    (a) Doodh ke saath (b) Khali pet, subah nashte se pehle (c) Raat ke khane ke saath (d) Iron ke saath
17. Carbimazole ka serious side effect –
    (a) Hypertension (b) Gum hyperplasia (c) Hirsutism (d) Agranulocytosis
18. Pregnancy me contraindicated –
    (a) Propranolol (b) PTU (c) Levothyroxine (d) Radioactive iodine

19. Diabetes insipidus ka DOC –
(a) Insulin (b) Thyroxine (c) Desmopressin (d) Oxytocin

20. Emergency contraception (levonorgestrel 1.5 mg) – kab tak –
(a) 1 month (b) 7 din (c) 24 hours only (d) 72 hours

? Level 2 – Priority & Application

21. Diabetic patient shaky, sweating, BS 52 mg/dl, conscious – nurse –
(a) Insulin do (b) Seedha IV 50 % dextrose (c) Kuch nahi (d) 15 g fast-acting sugar, 15 min baad recheck

22. Unconscious hypoglycaemic patient –
(a) IV 50 % dextrose / glucagon IM (b) Insulin (c) Wait (d) Mouth me juice

23. Steroid long-term le rahe patient ko advice –
(a) Jab chaho band (b) Raat ko lo (c) Achanak band nahi karna – taper (d) Khali pet lo

24. Steroid dene ka best time –
(a) Raat ko (b) Subah, khane ke saath (c) Dopahar (d) Khali pet

25. Carbimazole le rahe patient ko sore throat & fever –
(a) Dose badhana (b) Antibiotic khud lena (c) Ignore (d) Drug rok kar WBC count karwana

26. Alendronate le rahe patient ko –
(a) Khali pet, full glass paani, 30 min upright (b) Letkar lena (c) Khane ke saath (d) Doodh ke saath

27. Morning hyperglycaemia + raat 3 baje hypoglycaemia –
(a) Normal hai (b) Somogyi effect – evening insulin kam (c) Dawn phenomenon (d) Insulin badhao

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Long-term corticosteroid ka side effect NAHI hai –
(a) Hypoglycaemia (b) Osteoporosis (c) Hypertension (d) Infection risk

29. Assertion (A): DKA me insulin ke saath potassium monitor karte hain. Reason (R): Insulin potassium ko cells ke andar shift karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Order 10 units regular insulin; vial U-100 – kitna ml –
(a) 0.01 ml (b) 0.1 ml (c) 1 ml (d) 10 ml

Answer Key & Explanation

1-a Banting & Best 1921.
2-c 100 units/ml.
3-b Regular only.
4-a Glargine.
5-b NPH (isophane).
6-a Clear before cloudy.
7-c Abdomen.
8-c Lipohypertrophy.
9-c 2–8 °C.
10-a Hypoglycaemia.
11-a < 70 mg/dl.
12-c Metformin.
13-d Lactic acidosis.
14-d Sulfonylurea.
15-b Fluid retention.
16-b Empty stomach.
17-d Agranulocytosis.
18-d RAI – fetal thyroid destroy.
19-c Desmopressin.
20-d 72 hrs.
21-d Rule of 15.
22-a NPO – IV dextrose/ glucagon.
23-c Adrenal crisis – taper.
24-b Diurnal rhythm + GI protection.

25-d Agranulocytosis sign.
26-a Oesophagitis rokna.
27-b Somogyi.
28-a Steroids hyperglycaemia karte hain.
29-a Dono sahi, R reason hai.
30-b 10/100 = 0.1 ml.

9 Anticancer Drugs, Antidotes & Special Populations

Level 1 – Direct

1. Haemorrhagic cystitis –
   (a) Bleomycin (b) Doxorubicin (c) Vincristine (d) Cyclophosphamide
2. Cyclophosphamide bladder toxicity rokne ke liye –
   (a) Allopurinol (b) MESNA (c) Leucovorin (d) Dexrazoxane
3. Cardiotoxic anticancer drug –
   (a) Cisplatin (b) Doxorubicin (c) Methotrexate (d) Vincristine
4. Pulmonary fibrosis –
   (a) 5-FU (b) Vincristine (c) Cyclophosphamide (d) Bleomycin
5. Methotrexate toxicity ka rescue –
   (a) Leucovorin (folinic acid) (b) MESNA (c) Protamine (d) Vitamin K
6. Vincristine ki main toxicity –
   (a) Haemorrhagic cystitis (b) Ototoxicity (c) Peripheral neuropathy (d) Cardiotoxicity
7. Chemotherapy ki commonest dose-limiting toxicity –
   (a) Rash (b) Constipation (c) Bone marrow suppression (d) Alopecia
8. Most emetogenic drug –
   (a) Vincristine (b) Tamoxifen (c) Cisplatin (d) Bleomycin
9. Cytotoxic waste kis bag me –
   (a) Black (b) Yellow (c) Blue (d) Red
10. Cyclosporine ka side effect –
    (a) Orange urine (b) Nephrotoxicity & gum hyperplasia (c) Hypoglycaemia (d) Ototoxicity
11. Iron poisoning ka antidote –
    (a) BAL (b) Desferrioxamine (c) Penicillamine (d) EDTA
12. Lead poisoning –
    (a) Atropine (b) $CaNa_2EDTA$ (c) Naloxone (d) Desferrioxamine
13. Methaemoglobinaemia –
    (a) Methylene blue (b) Vitamin K (c) Flumazenil (d) Protamine
14. Methanol poisoning –
    (a) Naloxone (b) Fomepizole/ ethanol (c) NAC (d) Atropine
15. Cyanide poisoning –
    (a) Protamine (b) Hydroxocobalamin (c) Naloxone (d) Physostigmine
16. Thalidomide ka teratogenic effect –
    (a) Deafness (b) NTD (c) Ebstein's anomaly (d) Phocomelia
17. Lithium in pregnancy –
    (a) Gray baby (b) Kernicterus (c) Phocomelia (d) Ebstein's anomaly
18. Pregnancy me safe antibiotic –
    (a) Ciprofloxacin (b) Streptomycin (c) Amoxicillin (d) Tetracycline

19. Organogenesis (critical period) –
(a) 3–8 weeks (b) 12–16 weeks (c) 20–24 weeks (d) 36–40 weeks

20. Elderly me prescribing principle –
(a) Start low, go slow (b) High dose first (c) IV only (d) Polypharmacy best

? Level 2 – Priority & Application

21. Chemo patient ka temperature 38.5 °C, ANC 400 – nurse –
(a) Discharge (b) Neutropenic sepsis – turant inform, cultures, antibiotics as ordered (c) Paracetamol dekar wait (d) Ice pack only

22. Vesicant chemo extravasation – pehla step –
(a) Infusion continue (b) Infusion band, cannula se aspirate (c) Flush karo (d) Massage karo

23. Neutropenic patient ke room me –
(a) Fresh flowers allowed (b) Fresh flowers/ raw food allowed nahi (c) Visitors unlimited (d) Raw salad

24. Chemotherapy prepare karne se kaun bache –
(a) Night staff (b) Senior staff (c) Male staff (d) Pregnant staff

25. Anti-TNF biologic shuru karne se pehle screening –
(a) Lipid profile (b) Vision (c) HbA1c (d) Latent TB

26. Bachche ne mother ki iron goliyan kha li – antidote –
(a) Activated charcoal (b) NAC (c) Desferrioxamine (d) Naloxone

27. Kerosene poisoning me –
(a) Vomiting induce nahi karna (b) Gastric lavage mandatory (c) Ipecac do (d) Charcoal best

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Activated charcoal kis poisoning me useful NAHI –
(a) Iron (b) Phenobarbitone (c) Paracetamol (d) Theophylline

29. Assertion (A): Vincristine sirf IV deni chahiye. Reason (R): Intrathecal vincristine fatal hoti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Heparin Q. Warfarin R. Benzodiazepine S. Organophosphate – 1. Protamine 2. Vitamin K 3. Flumazenil 4. Atropine + 2-PAM
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-1, Q-2, R-3, S-4 (d) P-2, Q-1, R-3, S-4

Answer Key & Explanation

1-d Acrolein – haemorrhagic cystitis.
2-b MESNA.
3-b Doxorubicin cumulative dose.
4-d Bleomycin.
5-a Leucovorin.
6-c Neuropathy, constipation.
7-c Myelosuppression.
8-c Cisplatin.
9-b Yellow bag (cytotoxic label).
10-b Nephrotoxicity, gum hyperplasia.
11-b Desferrioxamine.
12-b EDTA.
13-a Methylene blue.
14-b Fomepizole.
15-b Hydroxocobalamin.
16-d Phocomelia.
17-d Ebstein's.
18-c Penicillins safe.
19-a 3–8 weeks.
20-a Start low, go slow.
21-b Febrile neutropenia emergency.
22-b Stop, aspirate.

23-b Infection source.
24-d Teratogenic risk.
25-d TB reactivation.
26-c Iron charcoal se bind nahi hota.
27-a Aspiration pneumonitis.
28-a Iron, lithium, alcohols charcoal se bind nahi hote.
29-a Dono sahi, R reason hai.
30-c Sahi match.

10 Emergency Drugs, High-Alert Drugs & Mixed PYQ

Level 1 – Direct

1. Eclampsia me DOC –
   (a) Phenytoin (b) Diazepam (c) Labetalol (d) Magnesium sulphate
2. $MgSO_4$ toxicity ka antidote –
   (a) Naloxone (b) Protamine (c) Calcium gluconate (d) Vitamin K
3. $MgSO_4$ toxicity ka pehla sign –
   (a) Hypertension (b) Knee jerk absent (c) Polyuria (d) Tachycardia
4. PPH ka first-line drug –
   (a) Methylergometrine (b) Oxytocin (c) Misoprostol (d) Carboprost
5. Hypertension me contraindicated oxytocic –
   (a) Misoprostol (b) Oxytocin (c) Methylergometrine (d) Tranexamic acid
6. Oxytocin ka high-dose side effect –
   (a) Water intoxication (b) Hyperkalaemia (c) Hypoglycaemia (d) Bradycardia
7. Lithium ka therapeutic range –
   (a) 0.1–0.2 mEq/L (b) 0.6–1.2 mEq/L (c) 2–3 mEq/L (d) 5–10 mEq/L
8. Digoxin ka therapeutic range –
   (a) 0.5–2 ng/ml (b) 5–10 ng/ml (c) 10–20 ng/ml (d) 20–40 ng/ml
9. Warfarin ka target INR (most conditions) –
   (a) 1 (b) 2–3 (c) 5–6 (d) 7–8
10. High-alert drug –
    (a) Vitamin C (b) Paracetamol tablet (c) ORS (d) Potassium chloride concentrate
11. PSVT me drug (vagal manoeuvre ke baad) –
    (a) Adrenaline (b) Adenosine (c) Digoxin IV fast (d) Atropine
12. Hyperkalaemia me heart ko stabilise karta hai –
    (a) Calcium gluconate (b) Insulin (c) Furosemide (d) Salbutamol
13. Isotonic fluid –
    (a) 0.45 % NaCl (b) 0.9 % NaCl (c) 3 % NaCl (d) 10 % dextrose
14. Hypertonic saline –
    (a) 3 % NaCl (b) 5 % dextrose (c) 0.9 % NaCl (d) Ringer lactate
15. Blood ke saath chalne wala ekmatra fluid –
    (a) Normal saline (b) DNS (c) 5 % dextrose (d) Ringer lactate
16. Black stools –
    (a) Methylene blue (b) Doxorubicin (c) Iron/ bismuth (d) Rifampicin
17. "-pril" suffix –
    (a) Statin (b) ARB (c) β-blocker (d) ACE inhibitor
18. "-olol" suffix –
    (a) PPI (b) CCB (c) ARB (d) β-blocker

19. Nitrates + sildenafil –
(a) Bradycardia only (b) Hypertension (c) Hypoglycaemia (d) Severe hypotension

20. MAOI + cheese –
(a) Hypotension (b) Hypoglycaemia (c) Diarrhoea (d) Hypertensive crisis

? Level 2 – Priority & Application

21. $MgSO_4$ ki next dose tab dete hain jab –
(a) Knee jerk absent (b) Urine 10 ml/hr (c) Knee jerk present, RR ≥ 12, urine ≥ 30 ml/hr (d) RR 8/min

22. Lithium le raha patient ko diarrhoea & vomiting – risk –
(a) Lithium toxicity (dehydration) (b) Lithium level girega (c) Koi risk nahi (d) Hypertension

23. Oxytocin infusion par contractions har 1.5 min, 90 sec lambi – nurse –
(a) Continue (b) Bolus do (c) Infusion band karke left lateral, O_2, inform (d) Rate badhao

24. Insulin aur heparin jaise high-alert drugs ke liye –
(a) Bina label (b) Abbreviation "U" likhna (c) Independent double-check (d) Verbal order enough

25. Patient warfarin + aspirin le raha – risk –
(a) Hypoglycaemia (b) Hypertension (c) Clotting (d) Bleeding

26. "Hold the drug" situation NAHI hai –
(a) Metoprolol – BP 80/50 (b) Paracetamol se pehle temperature 38.5 °C (c) Morphine – RR 10 (d) Digoxin – pulse 50

27. Anaphylaxis me adrenaline IM ka preferred site –
(a) Deltoid (b) Gluteal (c) Anterolateral thigh (d) Abdomen

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

28. Dopamine 400 mg in 250 ml; order 5 µg/kg/min, weight 80 kg – ml/hr –
(a) 7.5 (b) 15 (c) 30 (d) 60

29. Assertion (A): KCl kabhi IV push nahi dena chahiye. Reason (R): Rapid potassium se cardiac arrest ho sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match (urine colour): P. Rifampicin Q. Metronidazole R. Methylene blue S. Doxorubicin – 1. Orange-red 2. Dark brown 3. Blue-green 4. Red
(a) P-2, Q-1, R-3, S-4 (b) P-1, Q-3, R-2, S-4 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-2, R-3, S-4

✓ Answer Key & Explanation

1-d $MgSO_4$ (Pritchard/ Zuspan).
2-c 10 % calcium gluconate 10 ml IV.
3-b Loss of patellar reflex.
4-b Oxytocin 10 IU IM.
5-c Methylergometrine – vasoconstriction.
6-a ADH-like – water intoxication.
7-b 0.6–1.2.
8-a 0.5–2 ng/ml.
9-b 2–3.
10-d KCl concentrate.
11-b Adenosine 6 mg rapid push.
12-a Calcium gluconate.
13-b NS 0.9 %.
14-a 3 %.
15-a NS.
16-c Iron/ bismuth.
17-d ACEI.
18-d β-blocker.
19-d Severe hypotension.
20-d Tyramine – hypertensive crisis.

21-c Continue criteria.
22-a Dehydration → lithium retention.
23-c Hyperstimulation – stop oxytocin.
24-c Double-check.
25-d Bleeding.
26-b Fever me paracetamol hi dena hai – hold nahi.
27-c Anterolateral thigh.
28-b 400 mg/250 ml = 1600 µg/ml; 5 × 80 = 400 µg/min × 60 = 24,000 µg/hr ÷ 1600 = 15 ml/hr.
29-a Dono sahi, R reason hai.
30-d Sahi match.

11 GSSSB Pattern Mock Test – 100 MCQs (Full Pharmacology Syllabus)



Mock test ke rules

Time: 90 minutes · **Marking**: +1 correct, −0.25 wrong · **Score guide**: 80+ Excellent · 65–79 Good · 50–64 Weak topics revise karo · < 50 Notes dobara padho.

? Mock Test – Part A (1–50)

1. Drug ka absorption, distribution, metabolism, excretion –
(a) Posology (b) Pharmacogenetics (c) Pharmacodynamics (d) Pharmacokinetics

2. Bioavailability 100 % –
(a) IV (b) Rectal (c) Oral (d) IM

3. Sublingual route ka fayda –
(a) Liver metabolism ↑ (b) Slow action (c) First-pass bypass (d) Sirf bachchon me

4. Enzyme inducer –
(a) Ketoconazole (b) Erythromycin (c) Cimetidine (d) Phenytoin

5. Steady state –
(a) 1 half-life (b) 2 days always (c) 4–5 half-lives (d) 10 half-lives

6. Schedule X drugs –
(a) Ayurvedic (b) Narcotic/ psychotropic, strict record (c) Vitamins (d) OTC

7. IM site in infant –
(a) Deltoid (b) Ventrogluteal only (c) Dorsogluteal (d) Vastus lateralis

8. Intradermal angle –
(a) 10–15° (b) 45° (c) 60° (d) 90°

9. Ear drops adult –
(a) Seedha (b) Pinna up & back (c) Down & forward (d) Down & back

10. Drip rate: 1500 ml in 12 hrs, drop factor 20 – drops/min –
(a) 21 (b) 42 (c) 63 (d) 84

11. Order 75 mg; stock 50 mg/ml – ml –
(a) 0.5 (b) 1.5 (c) 2.5 (d) 3

12. Young's rule – 4 yr child, adult dose 500 mg –
(a) 100 mg (b) 125 mg (c) 200 mg (d) 250 mg

13. Myasthenia gravis drug –
(a) Pyridostigmine (b) Vecuronium (c) Atropine (d) Propranolol

14. OP poisoning pupils –
(a) Unequal (b) Normal (c) Dilated (d) Pin-point

15. Atropinisation ka end point –
(a) Salivation (b) Bradycardia (c) Pin-point pupils (d) Dry chest/ clear secretions

16. Anaphylaxis drug of choice –
(a) Antihistamine (b) Hydrocortisone (c) Adrenaline IM (d) Salbutamol

17. Septic shock vasopressor –
(a) Adenosine (b) Noradrenaline (c) Dobutamine (d) Isoprenaline

18. β-blocker contraindication –
(a) Hypertension (b) Asthma (c) Angina (d) Thyrotoxicosis

19. Captopril/ enalapril ka cough mechanism –
(a) Histamine ↓ (b) Infection (c) Mucus ↑ (d) Bradykinin ↑

20. Pregnancy HTN safe –
(a) Atenolol long-term only (b) Enalapril (c) Losartan (d) Labetalol

21. Nitroprusside infusion –
(a) Fast bolus (b) Oral (c) Light se protect (d) IM

22. Digoxin hold if apical pulse –
(a) < 90/min (b) < 60/min (c) < 100/min (d) > 60/min

23. Digoxin toxicity visual sign –
(a) Diplopia only (b) Yellow-green halos (c) Night blindness (d) Red vision

24. Furosemide fast IV –
(a) Constipation (b) Ototoxicity (c) Hyperkalaemia (d) Hypertension

25. K-sparing diuretic side effect –
(a) Ototoxicity (b) Hyperkalaemia (c) Hypokalaemia (d) Hypoglycaemia

26. Heparin antidote –
(a) Vitamin K (b) Protamine (c) Desferrioxamine (d) Flumazenil

27. Warfarin monitoring –
(a) Bleeding time (b) aPTT (c) Platelets (d) INR

28. Heparin SC site –
(a) Abdomen (2 inch away from umbilicus) (b) Gluteal (c) Deltoid (d) Forearm

29. Streptokinase – repeat nahi kyunki –
(a) Short acting (b) Oral only (c) Antigenic (antibodies) (d) Costly

30. Iron absorption badhata hai –
(a) Tea (b) Milk (c) Vitamin C (d) Antacid

31. KCl IV maximum rate (peripheral, usual) –
(a) 10 mEq/hr (b) 40 mEq/min (c) 50 mEq/hr (d) 100 mEq/hr

32. Blood unit complete karna –
(a) 4 hrs ke andar (b) 8 hrs (c) 12 hrs (d) 24 hrs

33. Acute asthma –
(a) Cromolyn (b) Nebulised salbutamol (c) Theophylline oral (d) Montelukast

34. ICS ke baad –
(a) Paani na piyo (b) Inhaler wash (c) Letna (d) Mouth rinse

35. Theophylline toxicity –
(a) Constipation (b) Seizures/ arrhythmias (c) Bradycardia (d) Hypoglycaemia

36. Montelukast best time –
(a) Subah (b) Khane se pehle (c) Bedtime (d) Dopahar

37. Non-sedating antihistamine –
(a) Hydroxyzine (b) CPM (c) Fexofenadine (d) Promethazine

38. PPI best time –
(a) Before breakfast (b) With milk (c) After dinner (d) Bedtime

39. Antacid aur tetracycline –
(a) Toxicity ↑ (b) Chelation – gap rakho (c) Koi effect nahi (d) Synergism

40. Chemo-induced vomiting –
(a) Loperamide (b) Ondansetron (c) Bisacodyl (d) Lactulose

41. Metoclopramide young female – acute dystonia ka Rx –
(a) Adrenaline (b) Promethazine/ benztropine IM (c) Haloperidol (d) More metoclopramide

42. Lactulose ka use –
(a) Asthma (b) Hepatic encephalopathy (c) Ulcer (d) Diarrhoea

43. Liquid paraffin side effect –
(a) Ototoxicity (b) Gum hyperplasia (c) Lipoid pneumonia (d) Hyperkalaemia

44. Laxative contraindicated –
(a) Intestinal obstruction (b) Elderly (c) Pregnancy (d) Constipation

45. Morphine hold if RR –
(a) < 20/min (b) < 12/min (c) < 24/min (d) > 12/min

46. Naloxone –
(a) Anticholinergic (b) Opioid antagonist (c) BZD antagonist (d) Opioid agonist

47. Paracetamol antidote –
(a) N-acetylcysteine (b) Atropine (c) Flumazenil (d) Naloxone

48. Aspirin child –
(a) Kernicterus (b) Phocomelia (c) Gray baby (d) Reye's syndrome

49. Phenytoin IV –
(a) IV push fast (b) IM preferred (c) Dextrose (d) NS only, ≤ 50 mg/min

50. Valproate pregnancy –
(a) Deafness (b) Phocomelia (c) Neural tube defect (d) Ebstein's

? Mock Test – Part B (51–100)

51. Status epilepticus first –
(a) Lithium (b) Lorazepam IV (c) Carbamazepine (d) Phenytoin oral

52. Levodopa + carbidopa –
(a) Peripheral decarboxylation rokna (b) Constipation (c) Hypotension rokna (d) Sedation

53. BZD antidote –
(a) Physostigmine (b) Atropine (c) Naloxone (d) Flumazenil

54. Lithium toxicity ↑ –
(a) Exercise (b) Dehydration/ low salt (c) High salt (d) Fluids ↑

55. NMS ka drug –
(a) Haloperidol (b) Succinylcholine (c) Dantrolene/ bromocriptine (d) Lithium

56. Clozapine monitoring –
(a) INR (b) WBC/ ANC (c) Lithium level (d) LFT only

57. MAOI ke saath avoid –
(a) Water (b) Milk only (c) Cheese/ tyramine (d) Rice

58. Malignant hyperthermia trigger –
(a) Ketamine only (b) Paracetamol (c) Succinylcholine/ halothane (d) Lignocaine

59. Ketamine –
(a) Muscle relaxant (b) Dissociative anaesthesia, ↑ BP (c) Hypotension (d) Bronchospasm

60. Spinal anaesthesia headache –
(a) Flat lying + fluids (b) Walking (c) Fluid restriction (d) Sitting

61. Penicillin first dose ke baad observe –
(a) 2 min (b) 5 hrs (c) 24 hrs (d) 30 min

62. RF prophylaxis –
(a) Cotrimoxazole (b) Ceftriaxone daily (c) Benzathine penicillin 21 days (d) Amoxicillin weekly

63. Aminoglycoside toxicity –
(a) Hepatotoxic (b) Teeth staining (c) Cardiotoxic (d) Ototoxic & nephrotoxic

64. Teeth staining –
(a) Cefixime (b) Penicillin (c) Erythromycin (d) Tetracycline

65. Gray baby –
(a) Chloramphenicol (b) Tetracycline (c) Amoxicillin (d) Gentamicin

66. Tendon rupture –
(a) Fluoroquinolone (b) Aminoglycoside (c) Macrolide (d) Penicillin

67. Metronidazole taste –
(a) Bitter-sweet (b) Metallic (c) No taste (d) Sweet

68. Vancomycin infusion time –
(a) 5 min (b) 15 sec (c) IV push (d) ≥ 60 min

69. Scrub typhus DOC –
(a) Doxycycline (b) Cotrimoxazole (c) Penicillin (d) Gentamicin

70. INH neuropathy prevention –
(a) Pyridoxine (b) Vitamin C (c) Thiamine (d) Folic acid

71. Ethambutol –
(a) Optic neuritis (b) Deafness (c) Gout (d) Orange urine

72. Rifampicin –
(a) Gum hyperplasia (b) Gout (c) Orange urine, enzyme inducer (d) Optic neuritis

73. Amphotericin B diluent –
(a) Ringer lactate (b) Normal saline (c) Water (d) 5 % dextrose

74. Influenza (H1N1) drug –
(a) Zidovudine (b) Acyclovir (c) Oseltamivir (d) Ribavirin only

75. HIV first-line (NACO) –
(a) Acyclovir (b) Nevirapine alone (c) AZT alone (d) TLD

76. Zidovudine side effect –
(a) Optic neuritis (b) Nephrotoxicity (c) Anaemia (d) Gout

77. Vivax radical cure –
(a) Artesunate (b) Quinine (c) Chloroquine 3 days only (d) Primaquine 14 days

78. Severe malaria –
(a) Injectable artesunate (b) Chloroquine oral (c) Doxycycline alone (d) Primaquine

79. Quinine side effect –
(a) Gum hyperplasia (b) Orange urine (c) Hyperglycaemia (d) Cinchonism & hypoglycaemia

80. Albendazole dose (> 2 yrs) –
(a) 1 g (b) 100 mg BD 3 days (c) 200 mg × 7 days (d) 400 mg single

81. Only IV insulin –
(a) NPH (b) Detemir (c) Regular (d) Glargine

82. Insulin mixing –
(a) Cloudy before clear (b) Mix with glargine (c) Clear before cloudy (d) Shake vigorously

83. Insulin in-use vial at room temp –
(a) ~28 days (b) 6 months (c) 1 year (d) 2 days

84. Hypoglycaemia conscious –
(a) Water only (b) Salt (c) Insulin (d) 15 g fast sugar

85. Metformin stop before –
(a) Exercise (b) Sleep (c) IV contrast (d) Breakfast

86. SGLT2 inhibitor side effect –
(a) Hypokalaemia (b) Constipation (c) Weight gain (d) Genital infection/ UTI

87. Levothyroxine –
(a) With iron (b) With antacid (c) Empty stomach morning (d) At night with milk

88. PTU used in –
(a) Gout (b) 1st trimester & thyroid storm (c) Hypothyroidism (d) Diabetes

89. Steroid withdrawal –
(a) Alternate weeks (b) Taper gradually (c) Stop abruptly (d) Double then stop

90. Steroid long-term –
(a) Weight loss (b) Osteoporosis (c) Hypotension (d) Hypoglycaemia

91. Alendronate –
(a) Lie down after (b) With milk (c) Upright 30 min (d) Bedtime

92. Acute gout –
(a) Allopurinol start (b) Aspirin low dose (c) Colchicine/ NSAIDs (d) Probenecid

93. Haemorrhagic cystitis prevention –
(a) Leucovorin (b) MESNA + hydration (c) Vitamin K (d) Dexrazoxane

94. Doxorubicin –
(a) Gum hyperplasia (b) Ototoxicity (c) Gout only (d) Cardiotoxicity

95. Methotrexate (RA) dose –
(a) Monthly IV (b) Weekly (c) Daily (d) Hourly

96. Iron poisoning –
(a) BAL (b) Penicillamine (c) EDTA (d) Desferrioxamine

97. Methaemoglobinaemia –
(a) Vitamin K (b) NAC (c) Naloxone (d) Methylene blue

98. Cyclosporine side effect –
(a) Hypoglycaemia (b) Ototoxicity (c) Nephrotoxicity & gum hyperplasia (d) Orange urine

99. Category X –
(a) Insulin (b) Isotretinoin (c) Penicillin (d) Paracetamol

100. Elderly inappropriate drug list –

(a) Braden (b) Glasgow (c) Beers criteria (d) APGAR

Answer Key – Mock Test (with explanation)

1-d ADME.
2-a Seedha circulation.
3-c Fast onset.
4-d Phenytoin inducer.
5-c 4–5 t½.
6-b Strict control.
7-d Well developed muscle.
8-a ID shallow.
9-b Adult canal.
10-b 1500×20/720 = 41.7.
11-b 75/50.
12-b 4/16 × 500.
13-a Anticholinesterase.
14-d Muscarinic – miosis.
15-d Secretions dry.
16-c Adrenaline first.
17-b First-line.
18-b Bronchospasm.
19-d Bradykinin.
20-d Labetalol/ methyldopa/ nifedipine.
21-c Photosensitive.
22-b Adult < 60.
23-b Xanthopsia.
24-b Ototoxic.
25-b Spironolactone.
26-b Protamine.
27-d PT/INR.
28-a Abdomen.
29-c Allergy.
30-c Ascorbic acid.
31-a Slow dilute.
32-a 4 hrs.
33-b SABA.
34-d Thrush.
35-b Narrow TI.
36-c Night.
37-c 2nd gen.
38-a Empty stomach.
39-b Absorption ↓.
40-b 5-HT3.
41-b Anticholinergic.
42-b ↓ ammonia.
43-c Aspiration.
44-a Perforation.
45-b Respiratory depression.
46-b Antagonist.
47-a NAC.
48-d Avoid.
49-d Precipitates in D5.
50-c NTD.
51-b BZD.
52-a DDC inhibitor.
53-d Flumazenil.
54-b Retention.
55-c NMS Rx.
56-b Agranulocytosis.
57-c Hypertensive crisis.
58-c Triggers.
59-b Dissociative.
60-a PDPH.
61-d Anaphylaxis window.
62-c 21 days.
63-d 8th nerve.
64-d < 8 yrs avoid.
65-a Neonate.
66-a Achilles.
67-b Metallic.
68-d Red man.
69-a Doxy.
70-a B6.
71-a Vision check.
72-c Orange secretions.
73-d D5W.
74-c Within 48 hrs.
75-d TLD.
76-c Bone marrow.
77-d Hypnozoites.
78-a Artesunate.
79-d Cinchonism.
80-d 400 mg.
81-c Regular.
82-c RN.
83-a 28 days.
84-d Rule of 15.
85-c Lactic acidosis.
86-d Glucosuria.
87-c Absorption.
88-b PTU.
89-b Adrenal crisis.
90-b Bone loss.
91-c Oesophagitis.
92-c Allopurinol not in acute.
93-b MESNA.
94-d Cardiomyopathy.
95-b Daily = fatal.
96-d DFO.
97-d Methylene blue.
98-c Calcineurin inhibitor.

99-b Teratogen.
100-c Beers.

— Pharmacology · GSSSB Staff Nurse Preparation · Padhte raho, revise karte raho! —